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Medico Legal Tip #1*

Doctors often mistakenly believe that patients who leave against medical advice (LAMA) do not need a discharge summary.

Legal Obligation:

All patients have the right to receive their treatment details, reports, and a discharge summary, regardless of whether they continue treatment or leave against medical advice.

Special Attention Required:*

Extra caution is necessary for patients leaving against medical advice. These patients are more likely to experience complications, and may even deteriorate or die during transport to another hospital.

Legal Defense:

The discharge summary is a crucial document that can defend doctors in a court of law. It must be provided at the time of discharge, even if the patient is leaving against advice.

Document Patient Condition:

The discharge summary should clearly mention any hemodynamic instability, such as unstable vitals, inotropic support, oxygen support, etc.

It should also document that the patient's attendants have been informed about the need for continued care, the risks of discontinuation, and transportation risks, despite which they chose to leave.

Avoid Suspicion:

Not providing a discharge summary can create suspicion and increase the likelihood of legal action by patient. It may appear to the judge that the doctor is attempting to hide potential negligence.

Best Practice:

A meticulously prepared discharge summary for LAMA patients can serve as a vital defense if a legal case is filed.

Medico Legal Tip #2

Blanket consent

◆ Scenario: It's common for an operating surgeon to start with one diagnosis and discover another condition requiring a different procedure during surgery. For example, appendicitis might turn out to be intussusception, or cholecystitis might be found to be pyelonephritis.

◆ Legal Principle: While it makes medical sense to proceed with the necessary new procedure to save time and money, but the law is clear: consent for one procedure cannot be used as consent for a different procedure unless it's a life-saving emergency.

◆ Case Reference: This principle was clearly explained in the Supreme Court judgment *Samira Kohli vs Dr. Prabha Manchanda & Anr* on January 16, 2008. In this case:

- A woman was scheduled for a diagnostic laparoscopy.
- She was found to have extensive endometriosis.
- A hysterectomy was performed under the same anesthesia.
- The court found the doctor negligent for not taking consent for the hysterectomy and awarded compensation.

◆ Examples:

✗ One cannot perform orchidopexy and phimosis with only orchidopexy consent unless consent is specifically taken for both.

✗ One cannot take consent for a diagnostic laparotomy and then operate for a torsion ovarian cyst.

✗ One cannot take consent for intubation and then insert a central line with the same consent.

◆ Recommendation: If a new procedure is required, doctors should:

- Take new consent from close relatives.
- For major surgeries like orchidectomy, hysterectomy, or oophorectomy, it is advisable to wait for the patient to come out of anesthesia and give consent, even though it may seem medically incorrect.
- Remember, for adults, proxy consent is not valid, and they may not be willing to undergo the procedure once they are out of anesthesia.

Medico Legal Tip #3

***Patient Admission and Financial Constraints ***

Many times, patients get admitted initially to our hospital. However, after a few days of treatment, they may find the private hospital charges unaffordable and request a referral to a government hospital.

Routine Discharge Summary Notes

In such scenarios, it is routine for the doctor to write in the discharge summary that the patient was referred to a government hospital due to non-affordability or was discharged against medical advice because of financial constraints .

Potential Risks During Transfer

Imagine that the patient dies on the way to the government hospital or immediately after reaching the government hospital and patient files a case against you in consumer court

*** Legal Faults***

1. Discontinuing Treatment: Discontinuing a treatment that has already been started is strictly against the law. 
2. Referring Patients Due to Fees:
Referring patients because they are unable to pay the fees is also a legal issue. 

***Patient's Perspective* :**

- When a patient or their attendants file a case, they will never admit requesting a referral to a government hospital.
- Instead, they may claim that the doctors stopped treatment due to the non-payment of hospital fees, resulting in their forced discharge to government hospital.

Documentation Challenges:

- Patients will have documented discharge letters stating discharge due to financial issues and initial bills, which they paid to the hospital when treatment was going on
- Legally, discontinuing treatment due to non-payment is against the law, even if it seems impractical.

*** Legal Implications* :**

- Documenting that a patient was referred due to inability to pay can be legally damaging.
- The principle of res ipsa loquitur (the thing speaks for itself) may apply, where the doctor's documentation could be seen as an admission of discontinuing treatment for non-payment.

Pravat Kumar Mukherjee vs. Ruby General Hospital & Ors (25 April, 2005)

In this case, a patient was initially taken to a private hospital after a road traffic accident (RTA) and later referred to a government hospital. The court addressed the critical issue of whether doctors can insist on payment when a patient's life is at risk.

Observation made by judges :

- The judge questioned, “Can doctors insist and wait for money (fees) when death is knocking the doors of the patient?”
- The obvious answer provided was, “Recovery of fee can wait - but not the death nor the treatment for trying to save the life.”
- Treatment which was started and later withdrawn can not be justified on ANY GROUND

This case highlights the ethical and legal obligations of medical professionals to prioritize life-saving treatment over financial considerations, and *Rs 1.3* crore was awarded as compensation.

Legal Analogies and Implications of Discontinuing Treatment Due to Non-Payment

Let me try to convince you with an example

Example of Firecracker Ban:

- Most state governments have banned bursting crackers after 10:00 PM. during Diwali.
- Despite this, many people burst crackers until midnight. However, this does not make it legal.
- If a neighbor complains to the police about bursting crackers after 10:00 p.m., you can be booked and fined.
- It is easy to prove in court if you have documented evidence stating you burst crackers at midnight.

 Legal Principle :

- Even if many people violate the law, the court will decide based on the law of the land when a complaint is given

- Similarly, discontinuing treatment due to financial reasons is against the law.

*

*Supreme Court Judgments** :

- Several previous Supreme Court judgments have made it clear that financial reasons can not justify referring a patient or discontinuing treatment.

⚠️ *Legal Risks:*

- Patients who initially request a government hospital referral may later claim in court that they were forced out of the hospital due to non-payment.
- Proper documentation using the recommended legally correct phrases can help protect against legal claims.

🚫 Never Mentioning Financial Constraints :

- Never write that a patient was referred to a government hospital due to financial constraints. This can be legally damaging and may lead to compensation claims if the patient dies during the transfer.

✍️ *Recommended Phrases for Referral Letters:*

- *Multidisciplinary Care:*

“Patient was referred for multidisciplinary care.”

- *Patient Request* :

“Patient requested a government hospital referral; hence referred after explaining all the risks.”

- *Attenders’ Preference* :

“Patient attenders want to continue treatment at the nearby government hospital.”

- *Best Interest of the Patient:*

“In the best interest of the patient and at the request of the attenders, the patient was referred to a higher center.”

✅⚖️ *Legal Tips for Referral Letters and Discharge Summaries*

- Never dig your own grave by documenting financial reasons for referrals in discharge summaries.
- Focus on medical justifications for any referrals or transfers.
- Ensure all documentation is clear, factual, and compliant with legal standards

Medico-Legal Tip #4** **

Two years ago, there was an incident reported in the paper where a sickness certificate was issued in the name of the then President of India. This certificate was obtained by an individual who misrepresented his identity to the doctor. There was no mention of any identification marks, patient signature, or thumb impression in the certificate, placing the doctor in a precarious position to justify the issuance.

When drafting a medical certificate (leave certificate, fitness certificate, wound certificate, etc.), it is imperative to verify the identity of the patient and document it on the certificate. This ensures that the certificate is issued to the individual who was actually examined.

Instances have occurred where an individual presents themselves under a false name to obtain a certificate, which is then used by another person.

For example, Ramesh, who has normal vision, may present himself as Abhishake to obtain a visual acuity certificate in the name of Abhishake, who is color blind. This certificate may then be used by Abhishake, who is visually impaired, to apply for a driving license or other jobs. This is just an example, and similar tactics can be employed to obtain bail, sickness benefits, insurance fraud, etc.

To mitigate such fraudulent activities, it is recommended to:

1.  Obtain Signature or Thumb Impression : Secure the signature or thumb impression of the individual on the certificate, along with the signature of the issuing doctor.
2.  Document Identification Marks: Record at least two identification marks of the individual on whose name the certificate is issued.
3.  Specify the Purpose: Clearly state the purpose of issuing the certificate. For instance, if a patient requests a certificate to join a fitness club, it should be noted: "This certificate was issued for the purpose of joining a fitness club."
4.  Include Time and Date: Always include the time and date of issuance of the certificate. Retain a photocopy of the certificate, if feasible, especially when issuing medico-legal certificates.

Adhering to these four points ensures legal compliance, and it

 Prevent Transfer : The certificate can not be transferred to another individual.

 Facilitate Future Verification: When called upon to verify the certificate in court after several years, having documented identification marks will facilitate the process. This is

crucial as the patient's physical appearance may change, or the details may be forgotten, given that cases can arise 5 to 10 years post-issuance.

This approach helps maintain the integrity and authenticity of medical certificates.

Medico Legal Tip #5

Clarification Regarding Consent in Medicolegal Cases_

 **What is an MLC Case?** :A Medico-Legal Case (MLC) is a case of ailment where investigation by law enforcement agencies is essential to determine the causes.

 **Concept Behind Police Intimation** : Intimating a case as an MLC is merely informing the police that an event has occurred which requires their attention. It is not a complaint or a case filing.

! Misconceptions Among Doctors : Many doctors mistakenly believe that by intimating an MLC, they are filing a complaint, initiating a case, or that the patient or themselves will be dragged to court for all MLC intimations.

 **What Happens After You Intimate ?** : Once the police receive an intimation, they do not immediately file a case or FIR. They will visit the patient, collect information, and conduct a preliminary enquiry. If they determine that a complaint and case filing are necessary, they will proceed. Otherwise, they will close the intimation. Cases like self-fall, snake bite, attempted suicide, or accidental consumption of poison by pediatric patients are often closed without further issues.

Let me give an example, if a patient with an attempted suicide is reported to the police, they will investigate to determine if the suicide attempt was due to external forces (e.g., dowry torture, physical abuse). If so, they will proceed with a complaint/FIR. If it is due to issues like depression, they will not proceed further.

 **Reassurance for Doctors** : Doctors should not be under the impression that they are filing a case making an FIR, or troubling patients by intimating an MLC. Similarly, doctors who intimate MLCs will not be unnecessarily troubled or called to court or the police station for all cases.

Consent and MLC : It is purely at the discretion of the attending doctor to decide when to label a case as a Medico-Legal Case (MLC). The request of the patient or the accompanying relatives or friends for registering or not registering the case as an MLC is irrelevant . Consent of the patient or family members is not required for registering a case as an MLC.

Even if you mistakenly report a non-MLC as an MLC, don't worry. The police will simply close the intimation once they determine that it is not an MLC, and no harm will be done to the doctors.

Patient or attenders consent has no role in making a case MLC.

Categories of Cases to be Labelled as MLC:

1. Cases of attempted abortion by an unauthorized person/criminal abortion.
2. Cases of snake bite, scorpion sting, animal bites, etc. 🐍🦂
3. Cases of unknown unconscious patients (unaccompanied by any relative). 🤔
4. Persons under police custody or judicial custody. 🚓
5. Cases of domestic violence. 🏠
6. Cases of child abuse. 🧒
7. Cases of human rights violations. 🚫
8. Brought dead cases. 🍖
9. Sudden death cases. 💔
10. Deaths occurring during any medical or surgical procedure. 🏥
11. Any other case not falling under the above categories but has legal implications or patients brought to the hospital under suspicious circumstances. 🤔

Golden Points to Remember :

1. No need for consent to make an MLC; attendants have no role in this. ❌📄
2. Making an MLC does not mean filing an FIR or giving a complaint. 📄
3. Doctors will not be punished if they mistakenly report a non-MLC as an MLC. 👍
4. The doctor's duty is just to intimate; further actions will be taken by the police based on a preliminary enquiry. 🚔

What should a doctor check before buying professional indemnity insurance?

1 Check the AOA : AOY Ratio — This decides how much you ACTUALLY get
“Your policy may say 1 Crore, but if the AOA:AOY ratio is 1:2, the company will pay only 50 Lakhs for a single claim.
If the court awards 80 Lakhs, you will pay the remaining 40 Lakhs from your own pocket.”
“Most doctors don't even know this ratio exists — but this is the clause that destroys them financially.”

2 Check if Vicarious Liability is Covered

“Many policies cover only the doctor.

But in real life, you will be sued even if the mistake is done by a nurse, technician, or junior, because of the legal principle of Vicarious Liability.”

“If your policy doesn't cover vicarious liability, you are standing naked in the courtroom.”

3 Check if Legal Defense Costs Are Included

“Hiring a good medico legal advocate today can cost more than the compensation claim itself.

If your policy doesn't cover legal defense, you will burn lakhs before the case even starts.”

4 Final Message

Indemnity insurance is your last line of defense.

Your first line of defense is preventing cases by learning the law.”

“Attend medico legal masterclasses specially designed for doctors.

Visit the website WWW.MEDICOLEGALAID.COM to register and learn the legal issues concerned with medical practice that can save your career.”

Understanding Consent and Legal Activities 🚫

Many doctors believe that obtaining consent allows them to do anything, but this is not legally accurate. Consent is not valid for illegal activities.

Let me clarify for you with a few examples

* Example 1: Selling Cocaine 🚫*

If I take written consent from you stating that you will not hold me responsible and then sell you cocaine, it does not make the act legal. Selling cocaine is illegal regardless of consent.

Example 2: Assisted Suicide ⚖️

Similarly, if an adult patient wants to commit suicide and provides written consent for it, and we supply poison to them, we are not legally protected. Assisting someone in committing suicide is illegal, regardless of their consent.

Key Concept: Consent and Legality 📄✍️

Consent is not valid for illegal activities. Even if you perform illegal activities with consent, they remain illegal. Now, apply this principle to medical consent.

📄✍️ Example of Legal Implications in Medical Practice

Termination of Pregnancy at 28 Weeks:

Even with the consent of the husband, wife, and parents, terminating a pregnancy at 28 weeks is illegal without court permission. Performing such an act can result in legal liability for the doctor, as termination at this stage is prohibited by law.

Importance of Understanding Legal Boundaries

Many doctors believe that obtaining consent protects them from legal repercussions. However, this is not always the case. In the above examples, if any relative later files a complaint about the termination of a 28-week pregnancy, the doctor can be held criminally negligent. The consent given by the family members will not be valid because the acts themselves are illegal.

Recommendations for Doctors

1. **Be Aware of Legal Limitations:** Ensure you are well-informed about what actions are legal and illegal in medical practice.
2. **Understand the Limits of Consent:** Just because you have obtained signatures does not mean you are authorized to perform any action on the patient.

Medicolegal tip # 7

"A Must-Know Weapon for Every Doctor : The Ultimate Legal Defence in Consumer Courts"

Under the law, if a patient has been negligent and their negligence has contributed to the damage, the defence of contributory negligence can be invoked by the doctor. In such situations, the compensation awarded will be reduced, taking into account the patient's share of responsibility for the damage.

It is important to remember that if a patient fails to follow a doctor's advice, and this failure causes their injuries or complications, it is possible to argue that the patient has been contributorily negligent. In extreme cases, the patient's conduct may be considered the sole cause of the damage.

Decided Case Laws :

1. **Devendar Singh v. Vivek Pal:**
The patient did not attend follow-up appointments and used prolonged medication on their own. The doctor was held liable for only limited compensation as the patient was found guilty of contributory negligence.
2. **Rohini Devi v. Dr. H. S. Chandavat:**
The patient did not attend follow-up appointments after surgery. The doctor was held not liable.

3. Manager, Martin Medical Centre v. E. V. Thomas :

The doctor provided first aid and treated an accident victim, later referring the patient to another hospital. The patient did not follow the doctor's advice, resulting in a reduced compensation amount due to the patient's contributory negligence.

Examples of Contributory Negligence 🚑:

1. Failure to provide the doctor with an accurate medical history or HIDING relevant medical history.
2. REFUSAL to take the suggested treatment.
3. Leaving the hospital AGAINST the doctor's advice.
4. NOT following post-operative advice.
5. Not adhering to proper PHYSIOTHERAPY protocols.
6. REFUSING TO get admitted to the ICU when advised.

The Importance of Documenting Contributory Negligence 📝:

Contributory negligence can serve as a strong defence in a court of law. However, it is crucial for doctors to meticulously document these instances, as this is often overlooked. Here are some examples to illustrate this point:

1. Missed Follow-Up Appointments:

If a doctor advises a patient to return after one week, but the patient comes back after 20 days, it is essential to document in the case or OPD sheet that the patient did not come for the follow-up as advised.

2. Incomplete Course of Antibiotics:

If a patient does not complete the prescribed course of antibiotics, it is mandatory to note in the OPD or IP slip that the patient did not follow the doctor's advice. 💊

3. Non-Adherence to Medical Advice:

If a patient does not follow dietary or physiotherapy advice, it is wise to mention in the follow-up notes that the patient did not adhere to the medical recommendations. 🍽️🏃

4. Self-Medication:

If a patient has self-medicated before consulting the doctor, it is important to document that the patient took self-medication and did not experience any benefit. 💊🤔

Key Takeaway 🛡️ :

The key message here is to always highlight instances of contributory negligence in your OPD or inpatient case sheets. The only way to prove contributory negligence is through proper documentation by the treating doctor, which many doctors fail to do.

Importance of This Message 📢:

Always be aware of contributory negligence. If there is any contributory negligence, **START YOUR NOTES IN THE OPD OR IPD SHEET BY MENTIONING IT AT THE BEGINNING.** Once documented, it becomes a legal record that can be used as a defence in the future. 🛡️

Remember: No documentation, no defence.

Medicolegal tip # 8

📝 Facing Modern Consent Challenges with Innovative Solutions

New Trends in Patient Claims ⚖️

A new trend has emerged where patients, even after signing the consent form, claim in court that the procedure was not properly explained and that they signed in a hurry. This has become a significant issue for doctors, as patients are using this tactic to seek compensation or avoid responsibility.

📅 17 Prior Informed Consent

The concept of consent has evolved from "informed consent" to "prior informed consent." Courts now expect doctors to inform patients about procedures well in advance, allowing them ample time to consider the information and make an informed decision. In some instances, consent obtained just half an hour before surgery has been deemed invalid, as the court felt the patient did not have sufficient time to contemplate the information.

🕒 Timing of Consent

To avoid such situations, it is advisable for doctors to take consent a few hours before the procedure for all elective surgeries. For example, if you are planning a hernia surgery, which is elective, the consent can be taken a day before the surgery when the

patient is admitted for pre-anaesthesia evaluation. This way, the patient cannot argue in court that there was no time to contemplate.

Consent Not Understood

Another issue raised by patients is that the consent was not explained properly and was in English, which they did not understand.

Patient's Handwritten Note

To avoid such litigations, it is better to ask the patient or their representative to write a few lines in their own language on the consent form, stating that the procedure and its complications have been explained to them. This handwritten note by the patient will prevent them from later claiming that the consent was not properly explained.

*Best Practices

1. Take Consent a Few Hours Before the Surgery:

This ensures that the patient cannot claim there was no time to contemplate.

2. Avoid Taking Consent Just Before the Procedure:

Unless it is an emergency, consent should not be taken immediately before the procedure.

3. Have the Patient Write a Few Sentences in Their Own Handwriting:

This will prevent the patient from claiming in court that the procedure was not explained to them.

By following these practices, doctors can effectively manage consent issues and ensure legal protection.

Medico Legal Tip #9

"Doctors, please STOP doing this Fraudulent Practices !" 🚚

Common Practice

Many doctors take signatures on blank papers before performing procedures, after explaining high risks, or during the hospital admission process. Doctors often forget to complete these blanks which often results in incomplete files ending up in the medical records department.

⚖️ Legal Implications

- **Court Cases:** If a patient files a case and the judge discovers a signature on a blank paper, it can significantly tarnish the doctor's image, suggesting an attempt to hide facts or cheat the patient. This can lead to the doctor being held liable and all consents being invalidated.
- **Evidence:** Relatives of patients sometimes take photos of the signed blank paper, which can be used against the doctor later.

👮 Case Example

A hospital was fined ₹1,00,000 just for taking a signature on a blank paper, even though medical negligence was not proved. This highlights the severe consequences of such practices.

📝 Recommendations

1. **Detail Before Signature:** Always write procedure details or high-risk counseling information before obtaining the patient's signature.
2. **Counter-Signatures:** The person taking the consent should also countersign the document.
3. **Check Case Sheets:** Doctors should check the case sheet at the time of discharge and fill up all blank sheets to avoid future legal issues.
4. **Complete Consent Forms:** Ensure the consent form is completely filled before it is sent to the medical records department.

🛡️ Conclusion

Obtaining consent on a blank paper can be considered fraudulent by the court. By following these guidelines, doctors can protect themselves from potential legal and ethical issues, ensuring better patient care.

Medico Legal Tip #10

🌟 " Make this simple change in your prescription to protect yourself." 🌟

👩‍⚕️ Introduction 👩‍⚕️

Medical practitioners face numerous challenges when it comes to drug reactions. Ranging from mild to severe, like Steven Johnson syndrome and even death. These situations often lead to medical negligence cases.

❓ The Gray Area of Allergy Testing ❓

The decision to conduct drug or allergy testing is complex. Courts have both supported and rejected the necessity of allergy testing depending on the individual case. This leaves a gray area for medical practitioners.

⚠️ The Risk of Allergic Reactions ⚠️

We, as doctors, know that all substances, regardless of previous usage, dosage, or route of administration (oral or intravenous), can potentially cause allergies, anaphylaxis, and even death. Intravenous drug administration is usually approached with caution, but oral medications given in outpatient settings are often not suspected to cause allergies.

🔍 Legal and Medical Precautions 🔍

To ensure legal safety, it is prudent for doctors to ask about the patient's history of allergies and record it in the OPD sheet. This practice serves two purposes:

1. ***Legal Protection:** In case of any adverse reactions, the recorded history can demonstrate that the patient reported no known allergies, supporting the doctor's decision not to perform allergy testing.
2. ***Improved Patient Care:** Many patients may not disclose their allergy history unless specifically asked, so this practice can help in identifying potential risks.

📅 Implementation in Practice 📅

It is recommended that every doctor includes a section in the OPD sheet to record the patient's allergy history along with other information like weight and vitals.

Receptionists can ask this question during patient registration and mark the response before the patient sees the doctor.

This small step can enhance both legal safety and the quality of patient care.

Print this in your prescription along with other details:

History of allergy

Yes ☐ No ☐

Medicolegal tip: 11

Advertisements in Medical Practice

In today's competitive medical field, doctors often use advertisements to attract patients, claiming high success rates or 100% recovery. However, doctors are governed by the Consumer Protection Act and the Code of Ethics Regulations 2002, which prohibits making exaggerated claims about treatments and recovery.

Real Case Scenarios

1. Ophthalmologist's Claim:

Advertisement: Claimed 100% spectacle-free vision after laser eye surgery.

Reality: Surgery included discussing the chances of residual visual abnormalities, possibly requiring continued spectacle use.

Outcome: The patient went to court with the advertisement, dissatisfied with the results not matching the claims.

2. Gynecologist's Claim:

Advertisement: Claimed a 70% success rate for IVF treatments.

Reality: The actual success rate was only 15-20%, acknowledged by the patient in a consent form.

Outcome: When the treatment failed, the patient used the advertisement as evidence of misleading claims in court.

Legal Implications

The Legal Wrong:

Discrepancies between advertised success rates and those mentioned in consent forms were deemed misleading. The judge ruled these as misleading advertisements.

Consumer Protection Act :

This act allows consumers to seek redress against false and misleading advertisements, including compensation for loss and injury caused by such ads. In these cases, doctors created unmet expectations, leading to legal consequences.

Final Conclusion by the Judge 🏛️

The judge concluded that the doctors had published false claims to attract patients. The advertisements were deemed misleading, and both the doctors and the hospitals were found guilty of adopting unfair trade practices under the Consumer Protection Act. Consequently, the patients received compensation.

Advice for Medical Practitioners 📝

In this era of consumerism, doctors should:

1. **Adhere to Advertising Rules:**
Follow the rules under the Consumer Protection Act and the Code of Ethics Regulations 2002.
2. **Avoid Unsubstantiated Claims:**
Do not publish numerical claims in percentages without proper proof. Advertised success rates should be backed by previous work.
3. **Consistency in Communication:**
Ensure that the success rates in advertisements match those explained to patients during the consent process.
4. **Be Cautious with Advertisements:**
Understand that paper advertisements can be used as legal evidence by patients. Be diplomatic in advertisements and try not to assure results in percentages.

By adhering to these guidelines, doctors can avoid legal battles. 🛡️

🌟 The Case Sheet That Could Save You in Court: Every Detail Counts 🌟

🩺 The Problem

When critically ill patients arrive at hospitals, consultants often focus on urgent treatment. Meanwhile, junior doctors handle the case sheet writing part. Sadly, these notes are sometimes written casually and fail to reflect the patient's critical condition upon arrival.

⚠️ Why Poor Notes Can Cause Problems

In many cases, patients arrive on trolleys, semiconscious or even unresponsive. However, case sheets may be casually written like Vitals are normal, GCS is 15/15, CNS is within normal limits etc

❌ Such errors can create serious legal trouble later. Opposite Lawyers might argue that the patient was stable upon arrival, and died because of doctor negligence. This will be supported by normal admission vitals which are carelessly written in case sheet

👉 Why Good Documentation Is Critical

Doctors must remember:

🕒 Patients won't always be alive later to explain how critically ill they were when they arrived.

👥 Family members may not confirm the seriousness of the initial condition.

📝 What to Include in Admission Notes

Always record findings that highlight the patient's critical condition, such as: ❤️ Bradycardia or Tachycardia, 😓 Respiratory failure (e.g., low saturation), 🧠 Low GCS or confusion, ⚡ Hypotension, 🩸 Bleeding tendencies, 💧 Severe dehydration, 🚫 Very low urine output etc in ADMISSION NOTES

🛡️ Protecting Yourself Against Legal Claims

Documenting these details ensures a clear record that the patient was critically ill upon arrival. This acts as solid evidence to safeguard against negligence claims.

📌 Action Points for Doctors

1 Document Thoroughly: Case sheets must reflect the seriousness of the patient's condition. ✍️

2 Consultants, Stay Vigilant: Make it a habit to review the admission notes of all critically ill patients under your care. 👩👨

🚑 Accurate notes at the time of admission are your strongest legal defence!

Medicolegal tip: 12

🏥 Withholding a Dead Body for Lack of Payment

It is not uncommon for hospitals or nursing homes to refuse to release a deceased patient's body due to unpaid bills. However, the law is very clear and strict on this matter.

Legal Standpoint

Article 21 of India's Constitution guarantees all citizens the right to personal liberty and forbids inhumane treatment, according to this. Withholding a deceased person's body is not only illegal but also barbaric and torturous for both the individual detained and their relatives.

In *Devesh Singh Chauhan v. State & Ors. 2017 SCC OnLine Del 8130*, the Hon'ble Delhi High Court clearly pointed out that "Merely because the dues of the hospital treating the patient are outstanding, that certainly cannot be a reason to withhold the release of the patient"

As per Charter of Patient's Rights adopted by the National Human Rights Commission "caretakers have the right to the dead body of a patient who had been treated in a hospital and the dead body cannot be detailed on procedural grounds, including nonpayment/dispute regarding payment of hospital charges against wishes of the caretakers."

Steps that can be opted by Patients

If a hospital refuses to release a body due to unpaid bills, the relatives can take the following steps:

1. File a Complaint:  A simple written complaint to the nearby police station, district commissioner, or human rights commission is sufficient to get the body released.
2. Media Involvement:  Informing the media can also help, as they often create sensational news out of such incidents, putting pressure on the hospital to comply.

Financial Considerations for Hospitals

Hospitals have various ways to claim unpaid bills, such as:

- Filing a Civil Suit:  Against the patient's attendants for non-payment after handing over the dead body.
- Periodic Payments:  During daily counselling, so that when an untoward incident happens, there is less payment to make, reducing the burden on the patient's family.

Advice to Doctors

As a practicing doctor, I understand that this may seem regressive for the financial health of nursing homes. However, the law is very clear and strict: one cannot hold a dead body or a patient hostage for the payment of hospital bills. Doctors and hospital administrators must avoid getting into trouble by following this illegal path and getting into unnecessary trouble.

By adhering to these guidelines, doctors can avoid legal battles. 🛡️

Medicolegal tip : 13

Treating Patients Under 18 Years Old

Legal Requirement : Only individuals 18 years or older can give valid consent for medical treatment and procedures. For minors, consent must be provided by a parent or guardian. 📝

1. Common Confusion: College students in grades 10-12 staying in hostels often visit doctors without guardians. Many doctors treat these students or perform minor procedures without knowing the legal implications. ⚠️
2. Legal Fault: Consent from a minor is not legally valid. Any procedure performed without proper guardian consent can lead to legal complications. 🚫

Example : A 17-year-old pre-college student comes to you with a fever and a small abscess. You prescribe some antibiotics and perform an incision and drainage procedure. However, the patient develops a serious complication due to the antibiotics or progression of the disease. In such cases, it becomes very difficult for the doctor to prove in court that all the complications were explained to the patient, as the minor is not in a position to give consent. ⚖️ Later, the parents may argue in court that the child's consent was invalid, regardless of the doctor's good intentions. Convincing the judge of your intentions can be challenging and risky.

Tips for Examining Patients Under 18 Years Old:

1. Always ask for a guardian to accompany the child, whether it is a hostel warden, teacher, or a parent's friend who is available locally. 👤
2. Do not consider the patient's friend as a legal guardian because they are also minor, and their consent will not be valid. 🚫👤
3. You do not need to take consent when the situation is life-threatening or an emergency. Even if the patient has come alone, you can still proceed with the treatment, which does not require any consent. 🚑🏥

Best Practice :

- Always ensure a guardian is present to provide consent unless it's a life-threatening emergency.
- Consent from minors is not valid. ❌
- Do not treat minors without a guardian unless it's an emergency. 🚑

Medico Legal Tip 14

“ What Every Doctor Should Know About the Consumer Protection Act” 🏥

It's a common practice among doctors to treat economically disadvantaged patients without charging them, as part of our service to humanity. However, this is accompanied by neglect of proper documentation. Many times, we merely prescribe medications on a slip without writing proper notes. This casual attitude stems from the MISTAKEN belief that patients who receive free treatment are not eligible to file a case in consumer court, which is incorrect. 🚫

Understanding the Consumer Protection Act:

- 🏥 Hospitals/Clinics that charge some patients and exempt others due to their inability to afford services are GOVERNED by the Consumer Protection Act.
- 🏥 Only hospitals where ALL patients receive free treatment, irrespective of their paying capacity, are exempt from the purview of the Act.
- 📄 All legal requirements, such as taking proper history, obtaining consent, documenting thoroughly, and adhering to prescription protocols, should be followed in all cases, even if you are not charging patients.

Tips to remember

- ⚖️ Waiving charges for patients does NOT exempt healthcare providers from their legal responsibilities. Free patients can also be potential litigants.
- ⚖️ For all practical purposes, all cases have the right to file a case in consumer court except in very few instances.
- 📁 Doctors must stop this casual documentation habit when treating free cases, whether in OPD or IPD, and protect themselves from future litigations.

Medico Legal Tip 15

Doctors Beware: Non-Compliance with the POCSO Act Can Land You in Jail and Destroy Your Career!

 **Definition of Sexual Assault** Any sexual activity involving a child under 18 years of age is considered sexual assault, regardless of the child's consent or marital status.

 **Supreme Court Ruling** The Supreme Court of India, in its landmark judgment (*Independent Thought vs Union of India*, October 11, 2017), ruled that sexual intercourse with a girl below 18 years of age is considered rape, even if she is wife.

 **Mandatory Reporting** Doctors are legally required to report cases involving sexual activity with children under 18 years of age to the police. This includes:

- Situations where a married girl under 18 comes for Antenatal check up/delivery with her husband.
- Romantic relationships between students under 18 that result in physical intimacy.

To ensure confidentiality, doctors can report only the IP number and address of the child without disclosing the child's name.

 **Consequences of Non-Compliance** Failure to report such cases is a punishable offense under Section 21 of the POCSO Act:

-  For individuals: Imprisonment of up to 6 months, a fine, or both.
-  For institutions: Imprisonment of up to 1 year, along with a fine.

 **Important Reminder** In some cases, doctors may mistakenly assume that the consent of the child is valid and refrain from reporting the case. However, this is legally incorrect. Doctors must remember that they are not authorized to judge the validity of a child's consent.

If the girl later files a rape case—possibly due to personal issues—doctors who failed to report the case initially can face serious legal consequences.

 **Reminder for Compliance** It is critical for doctors to report all cases of sexual activity involving children under 18 to the police. This is a mandatory legal obligation under the POCSO Act, ensuring compliance with the law and protecting doctors from legal repercussions.

Medico-legal tip 16

Consent Gone Wrong: Why Doctors Are Losing in Court 🚨

Recently, doctors have faced new challenges in legal cases where patients, even after signing consent forms, claim the procedure was not explained properly or that they signed the forms hastily. Such claims often lead to demands for compensation or an attempt to avoid accountability, creating serious issues for medical professionals.

The Shift to "Prior Informed Consent"

The concept of consent has evolved from "informed consent" to "prior informed consent."

Courts now expect doctors to explain procedures well in advance, giving patients enough time to understand and make informed decisions.

Many doctors still take consent just a few minutes before surgery, which has been criticized by patient advocates. This practice is often seen as rushed and insufficient, and courts may declare such **last-minute consent invalid** as it does not provide adequate time for patients to consider the details.

Best Practices for Consent Timing 🕒

To avoid legal troubles, doctors should adopt timely consent practices:

Example :*

For procedures like hernia surgery, consent should be taken **a day before the operation,** ideally during the patient's admission for pre-anesthesia evaluation.

**"In non-emergency/semi-emergency cases, it is important to ensure that there is a sufficient **time gap* between obtaining consent and performing the surgery.*

Why It Matters ✨

Although it appears simple, this small change in practice will help to validate consent in court.

Medico-Legal tip: 17

Protect Yourself: Say NO to generic phrases in certificates*

Using generic phrases like "To Whomsoever It May Concern" in medical certificates is a common practice among doctors. However, this can lead to legal complications.

Case in Point:

◆ Recently, a psychiatrist issued a medical certificate with the heading "To Whomsoever It May Concern."" ◆

The certificate detailed the patient's medical condition, as requested by her husband, who claimed it was needed for seeking a second opinion.

◆ Later, the husband used the certificate in a family court case to apply for a divorce.
◆

The wife filed a case against the doctor for breaching privacy, and the court criticized the doctor for carelessly issuing a certificate without mentioning its purpose.

Such certificates can also be misused to :

👉 Avoid a court warrant.

👉 Claim money fraudulently without the doctor's knowledge.

Recommendations to Avoid Legal Issues:

Avoid generic headings 

Do not use vague phrases like "To Whomsoever It May Concerned.""

Instead, address the certificate to a specific individual, organization, or institution.

Clearly State the Purpose 

Always mention why the certificate is being issued.

For example :

"This certificate is issued for submission to [specific institution]."

"This certificate is issued for seeking a second opinion."

This ensures the certificate can not be used for any other purpose.

By following these steps, we can uphold ethical practices and avoid potential legal pitfalls.

Medicolegal Tip 18

Documenting This can Save Lakhs in Compensation

Under the Law: If a patient's negligence contributes to the damage, doctors can invoke the defence of *contributory negligence*. This can significantly reduce the compensation awarded, based on the patient's share of responsibility.

Decided Case Laws on Contributory Negligence:

1 Devendar Singh v. Dr xyz: The patient was advised to use Mitomycin-C for two weeks after surgery for Pterygium. They missed follow-up appointments and overused medication, leading to complete loss of vision. The doctor's liability was limited, and compensation was reduced from ₹10 lakhs to ₹50,000.

2 xyz v. E.V. Thomas: The doctor gave first aid for a knee injury and advised the patient to shift to a better-equipped hospital. The patient delayed this by 10 days, worsening the condition. The compensation was reduced to ₹10,000 as the patient was partly negligent.

Examples of Contributory Negligence:

1.  Hiding relevant medical history despite being asked.
2.  Refusing to take the suggested treatment.
3.  Leaving the hospital against medical advice (LAMA).
4.  Not following post-operative advice.
5.  Skipping physiotherapy protocols.
6.  Refusing ICU admission when advised.

Importance of Documenting Contributory Negligence:

Proper documentation of contributory negligence can be a solid defence in legal cases. Here's how doctors can ensure it's well-recorded:

1 Missed Follow-Up Appointments: Document if the patient does not follow up as advised, e.g., returning after 20 days instead of 1 week.

2 Incomplete Antibiotic Course: Record instances where patients fail to complete their prescribed course of antibiotics. 💊

3 Non-Adherence to Medical Advice: Note down if patients ignored dietary or physiotherapy recommendations. 🍴🏃

4 Self-Medication: Include details of any self-medication and delay in consulting doctor 💊🤔

Key Takeaway: Document to Defend!

Always highlight contributory negligence in your OPD or IPD notes. Start your notes with these details so that they become a part of the patient's permanent medical record. No documentation means no legal defence!

Final Reminder:

Be vigilant about contributory negligence and meticulous in documentation. These simple practices could protect you from significant liabilities in the future. 🛡️

Medicolegal Tip 19

" Journalist asking for Patient Details Under the RTI Act? What should i do? "

With increasing awareness of the Right to Information (RTI) Act, the public—including journalists, family members, advocates, and third parties—are approaching hospitals and doctors, demanding access to patient medical records under RTI.

However, medical professionals must be fully aware that they have the right to refuse such requests to protect patient privacy and uphold ethical medical practices. 🚫

🔴 Doctors must refuse to give patient details to third-party quoting following legal grounds in writing

- 📖 RTI Act, Section 8(1)(j): Personal information, including medical records, is exempt from disclosure unless a larger public interest justifies it.

- ⚖️ Legal Precedents:

The Central Information Commission (CIC) has ruled that medical records can not be disclosed under RTI unless:

- o The patient consents ✅
- o The information is required by law enforcement or a court order 🏛️

🔍 Doctors Must Be Vigilant Despite these legal protections, doctors often face pressure from outsiders misusing RTI provisions. It is crucial that healthcare providers firmly decline such requests, citing legal grounds for refusal. 💪

🛡️ **Medicolegal Tip 20**

📄 **Handling Document Requests in Medico-Legal Cases: A Practical Guideline for Doctors**

When a medico-legal case (MLC) is admitted to a hospital, the entire case sheet and related documents become legal property 📁, and the attending doctor assumes the role of custodian 👤. These records must be securely preserved 🔒 and submitted to the investigating authority whenever officially requested 🕵️.

⚠️ **Common Challenge Faced :**

Often, patient attenders or relatives insist on receiving copies of the full case sheet 📄, certificates 📄, or post-mortem reports 🧐. When the doctor refuses, they may start arguing 🗣️.

✅ **Recommended Approach for Conflict Avoidance :**

In such situations, doctors should ask the attenders to obtain a No Objection Certificate (NOC) 📄 from the police station 🚓 where the case is registered. This shifts the responsibility to the investigating officer 🕵️.

This strategic approach has two key advantages:

- In most instances (over 99%) ✅, the police will not issue an NOC ❌, and the attenders will not return.
- If the attenders do return with a valid NOC 📄 from the investigating officer 🕵️, the doctor is legally protected 🛡️ and can safely issue the requested documents 📄.

🔍 **Medical Follow-Up vs. Legal Documents**

Doctors should distinguish between documents necessary for ongoing medical care and those under legal scrutiny 📄. Information such as provisional diagnosis and lab investigation reports, required for the patient's medical follow-up, can be routinely provided ✅.

📌 **Final Note**

A polite insistence on an NOC 📄 can effectively diffuse confrontations 🚫 and safeguard healthcare professionals 👩👨

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🛡️ **Medicolegal Tip 20**

Penile surgery instead of leg surgery

You might have heard the recent news where doctors allegedly performed penile surgery instead of leg surgery. As a medical person, do you think this is even possible? Let's break it down as a doctor 🧠.

✅ While operating on the leg 🦵, doctors may have noticed phimosis and decided to do a circumcision in the same sitting—to save patient from undergoing one more anesthesia procedure 😴 and cost 💰. Medically, that might make sense.

⚖️ Legal Reality

But legally, that's not acceptable

🚫. Surgeons often begin with one diagnosis and find another 🔄—like suspected appendicitis on opening, turning into ovarian torsion 🌀. But even if it's medically correct, consent for one procedure can not be extended to another unless it's life-saving 🆘.

📖 Landmark Case

👤 Samira Kohli vs Dr. Prabha Manchanda The patient consented to a diagnostic laparoscopy, and the doctor performed a hysterectomy without fresh consent. Result: Supreme Court ruled negligence and awarded compensation 💰.

🔍 Few Real-Life Examples

If consent is taken for Orchidopexy and the surgeon can't do circumcision under the same consent

Consent taken for diagnostic laparoscopy, which can't be converted into laparotomy
❌

🛡️ What Should Doctors Do?

Always take fresh consent for a new procedure 📝✍️

For major surgeries (orchidectomy, hysterectomy, oophorectomy , enucleation, wait until the patient comes out of general anesthesia and explain the new procedure planned and proceed after consent.

🕒 Proxy consent is not valid for adults, which means relatives can't give consent on behalf of the patient

👤 Say NO to blanket consent

🔴 Every procedure needs specific, informed, written approval 📝

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Medico-legal advice

⚠️ ⚖️ Awareness on Section 376(2) of the Indian Penal Code, 1860

A Note to Medical Professionals

On August 2, 2025, former Janata Dal (Secular) MP Prajwal Revanna was sentenced to *imprisonment for the remainder of his natural life* by a special court, following his conviction for raping a domestic worker.

Many may wonder why this sentence was more severe than the usual 7–10 years of imprisonment seen in other rape cases. The reason lies in the application of Section 376(2) of the IPC, which deals with aggravated forms of rape—particularly when the perpetrator holds a position of power, trust, or authority, or when the circumstances are especially heinous.

🏥 Relevance to Doctors: Section 376(2)(e)

A specific subsection, Section 376(2)(e), states:

"Whoever, being on the management or on the staff of a hospital, commits rape on a woman in that hospital, shall be punished with rigorous imprisonment for a term which shall not be less than ten years but which may extend to imprisonment for life, which shall mean imprisonment for the remainder of that person's natural life."

This provision directly applies to hospital staff, including doctors, and carries severe punishment if convicted.

🚑 Why This Matters to Medical Professionals

Let me clarify: I am not implying that doctors are committing such crimes. However, there have been instances of false rape accusations made against doctors by patients. Therefore, it is crucial for male doctors to exercise extra caution during professional interactions to protect themselves and maintain trust.

Recommended Precautionary Measures

To safeguard against potential misunderstandings or false allegations, male doctors are advised to:

- Keep the chamber door open when speaking with female staff or students.
- Ensure the presence of a female nurse while examining female patients.
- Avoid one-on-one interactions with female students or paramedical staff in private settings.
- Install CCTV cameras in consultation rooms, corridors, and other feasible areas—ensuring patient privacy is respected while enhancing transparency.

Final Note

This message is intended to raise awareness among medical professionals about the legal implications of Section 376(2), especially 376(2)(e), and to encourage proactive steps to ensure safety, professionalism, and integrity in clinical practice.

Let's continue to uphold the highest standards of care—while also protecting ourselves through mindful conduct.

Medico-legal advice

What If a Patient Collapses from Anaphylaxis in Your Clinic?

Most of us give routine injections like Diclofenac, Emeset, or Paracetamol without a second thought. But what if something unexpected happens?

Imagine this: A patient develops anaphylaxis—a severe allergic reaction—and collapses, or even dies, after an injection or vaccination. The family files a medical negligence case.

In such cases, the legal focus isn't on *why* the reaction happened... But on *how well* the emergency was handled.

What the Court Will Look At

The key medico-legal questions are:

- Was the clinic equipped to handle emergencies?
- Did the doctor follow the proper anaphylaxis management protocol?
- Were life-saving drugs like Adrenaline, Avil, and Hydrocortisone given in time?
- Were essential emergency tools like an Ambu bag or intubation kit available?

When Is It Considered Negligence?

If your clinic is prepared and you act quickly using the right protocol, you may not be held responsible.

But if you're missing basic emergency medicines or equipment, it can be considered negligence, and compensation may run into lakhs of rupees.

Medico-Legal Lesson

- Anaphylaxis itself is not negligence.
- Failure to manage it due to lack of preparedness *is* negligence.

Medico-Legal Advice

No matter how small your clinic is, it's mandatory to keep:

- Emergency drugs to treat anaphylaxis
- Basic tools to help with resuscitation

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This provision directly applies to hospital staff, including doctors, and carries severe punishment if convicted.

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Let me clarify: I am not implying that doctors are committing such crimes. However, there have been instances of false rape accusations made against doctors by patients and getting bail under this is difficult. Therefore, it is crucial for male doctors to exercise extra caution during professional interactions to protect themselves and maintain trust.

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To safeguard against potential misunderstandings or false allegations, male doctors are advised to:

- Keep the chamber door open when speaking with female staff or students.
- Ensure the presence of a female nurse while examining female patients.
- Avoid one-on-one interactions with female students or paramedical staff in private settings.
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Let's continue to uphold the highest standards of care—while also protecting ourselves through mindful conduct

Doctor Kindness cost him his career

A patient was brought to the hospital with acute chest pain. An ECG was performed which was said to be normal, but while leaving the hospital premises, the patient suddenly passed away. The exact medical cause of death remained unknown.

However, the patient had life insurance covering accidental death. Out of goodwill, the attending doctors issued a death certificate stating that the patient had died due to an accidental fall from the hospital staircase—intended to help the dependents claim insurance money.

The Investigation

- The insurance company conducted its own independent inquiry before approving the claim.
- It was found that there was no definitive proof of an accidental fall, and no post-mortem was performed to confirm the cause of sudden death.
- Since no medico-legal case (MLC) was registered—a necessary step if an accident is involved—the insurance company rejected the claim.

- A formal complaint was lodged against the doctors before the Medical Council for issuing a misleading death certificate.

Consequences

Following an inquiry, the Medical Council found the doctor negligent in issuing a false death certificate. As a result, the doctor's name was erased from the medical register for 365 days—effectively suspending their practice.

Medicolegal Takeaway for Doctors

- 1 Do Not Alter Medical Diagnoses to assist patients or their families financially.
- 2 Follow Standard Medico-Legal Protocols—document accurately and truthfully.
- 3 Insurance Companies Conduct Thorough Investigations before approving claims, using forensic methods and medical evidence.
- 4 Unwarranted Deviations Can Lead to Serious Consequences for doctors, including legal troubles and professional suspension.

“Blank Signatures = Legal Landmines”

A Risky Routine

In many hospitals, it's common for doctors to obtain patient signatures on blank sheets during admissions, pre-procedure briefings, or high-risk counselling. These blanks often remain unfilled, eventually slipping into the medical records department unnoticed.

Legal Repercussions Are Real

- Courtroom Consequences: If a dispute reaches court and a signed blank paper is found, it casts doubt on the doctor's intentions—potentially implying fraud. Judges may and hold the doctor liable.
- Photographic Proof: Patient families sometimes photograph these blank-signed documents, turning them into damaging evidence later.

Real-World Fallout

In a recent ruling, a hospital was fined ₹1,00,000—not for clinical negligence, but simply for having a signature on a blank sheet.

Best Practices to Avoid Legal Mishaps

1. Write Before You Sign Include all necessary procedure details and counseling notes *before* the patient signs.

2. Add a Counter-Signature The staff member obtaining the consent should sign alongside to authenticate the process.
3. Audit Case Sheets at Discharge Before finalizing a discharge, ensure no blanks remain in the documentation.
4. Verify Consent Form Completion Prior to sending documents to the records department, double-check that all consents are properly and completely filled.

Safeguarding Trust and Integrity

Signing blank documents can be construed as fraudulent behavior. Doctors must adopt meticulous documentation habits to uphold legal compliance and ensure ethical patient care.

When Relatives Say “Don’t Tell the Patient...”

 It happens often. You're at the bedside, reviewing reports, when a worried family member pulls you aside and whispers:

“Doctor, please don’t tell him the diagnosis. He won’t be able to handle it.”

As doctors, we understand their fear. But what should we do? Are we allowed to hide the truth from the patient just because the relatives ask?

The Legal Rule:

Doctors are legally and ethically required to inform adult patients about their diagnosis, prognosis, and treatment—especially if the patient is conscious, oriented, and capable of understanding.

But there are exceptions where withholding information is legally justified: this is called as Therapeutic privilege

Its a situation where If telling the truth could cause serious harm to the patient —like triggering a panic attack, refusal of life-saving treatment, or emotional collapse—in such situation, the doctor may delay disclosure

Example :

An elderly patient is diagnosed with terminal-stage cancer. The attenders request that the diagnosis not be disclosed to the patient.

A postpartum mother is in the ICU, but her newborn has passed away in the NICU. The father requests that the mother not be informed about the baby’s death

An ICU patient with severe anxiety is recovering from a heart attack. Telling him another diagnosis immediately might trigger another cardiac event.

 What should you do as a doctor?

 Speak to close relatives and explain the situation.

 Document everything:

Why you're withholding the information.

Who was informed (names and signatures).

Medicolegal takeaway

Not disclosing information has a legal backing, which is called therapeutic privilege, which can be used by doctors whenever necessary but with proper documentation

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Right to Die with Dignity: What Doctors Must Know

 Legal Foundation

Under Article 21 of the Indian Constitution, the Right to Die with Dignity has been upheld by the Supreme Court in the landmark case: *Common Cause vs. Union of India* (2018). This ruling allows the withdrawal of life-sustaining treatment (WLST) in specific circumstances

 Common Misconception

Many doctors believe that the right to life is upheld, which is legally correct. However, implementing this right practically is not as easy as it seems. When a patient is terminally ill and their attendants request the withdrawal of life-sustaining treatment (WLST) using the right to die with dignity, some doctors think they can just take a signature on a routine case sheet and allow the patient to die by removing life support. This is 100% wrong and legally incorrect.

 Step-by-Step Legal Protocol

1. Primary Medical Board:

The hospital must form a primary medical board, including the treating physician and two independent medical experts who are not from the same department. These experts assess the patient's condition thoroughly and submit a written report based on their evaluation.

2. Secondary Medical Board :

The secondary medical board must be constituted by the District Collector and should comprise three independent medical experts. This board reassesses the patient's condition and submits a second report that confirms the findings of the primary medical board.

3. Judicial Magistrate of the First Class (JMFC):

Copies of the decisions of the Boards regarding WLST are to be submitted to the JMFC before giving effect to them. The JMFC will send copies to the Registrar of the High Court for record-keeping.

Key Takeaway for Doctors

I want to stress again that just taking a signature from the relatives on the case sheet is not enough legal documentation to remove life support, Although the Supreme Court has given the right to die with dignity under Article 21, it comes with many procedural formalities to safeguard against the misuse of this right.

Medico-Legal Tip: AYUSH Doctors in Allopathic Hospitals

 Judgement Quoted : Chandigarh Nursing Home and Another v. Sukhdeep Kaur, 09/09/2022

Key Issue

Appointment of Ayurvedic (AYUSH) doctors in allopathic nursing homes has become a common practice. Some State Governments have issued special orders allowing AYUSH doctors to prescribe allopathic medicines.

Vicarious Liability

If any negligence occurs (wrong diagnosis, incorrect prescription, etc.), responsibility falls on the hospital owner or appointing authority. The Courts have made it clear: the liability is vicarious and rests on the superior/hospital owner.

Medical Negligence & Unfair Trade Practice

- Prescribing allopathic medicines and misrepresenting them as traditional medicine = Unfair Trade Practice
- Not explaining side effects of allopathic medicines = Medical Negligence

Supreme Court Caution

Employing traditional medical practitioners (without allopathic competency) to treat emergency patients is gross negligence. In case of any unwanted outcome, hospital authorities are fully responsible.

Quoting directly from judgement (Chandigarh Nursing Home v. Sukhdeep Kaur, 09/09/2022) “Employing traditional medical practitioners who do not possess the required skill and competence to give allopathic treatment in hospitals and allowing them to treat emergency patients is gross negligence. The responsibility lies completely with the hospital authorities.”

👉 Medico-Legal Takeaway

If you appoint an AYUSH practitioner and they make an error, you cannot escape liability by saying they were authorised to prescribe medicines. Even if authorised, the hospital owner is vicariously liable for their actions.

⚠️ Medico Legal Tip: How to Handle Sudden Deaths in Hospital

👩⚕️ Sometimes a patient admitted for pneumonia, elective surgery, or even a newborn in the ward may die suddenly. The cause of death can be due to medical reasons — such as aspiration, myocardial ischemia, pulmonary embolism, or sudden infant death syndrome.

Relatives may argue that the patient died because of an injection or IV fluids. It is very difficult to explain the true medical cause of death to them, and these cases often end up in court with allegations of medical negligence. So how can doctors safeguard themselves from false allegations?

📌 Advice for doctors :

- Treat every unexplained sudden death as an MLC (Medico Legal Case).
- Inform police and advocate for postmortem to establish the medical cause of death.
- Preserve all records and samples: charts, monitoring strips, investigations, and specimens.
- Communicate clearly to relatives that a postmortem is needed to determine the exact cause.
- Remember: even an allegation of negligence is enough to make the case an MLC.
- Sudden unexplained death in hospital, where the cause is not known, is also enough indication to make the case an MLC.

🔍 Why this matters:

- Postmortem findings (food in the airway, histopathology of myocardial ischemia, pulmonary embolism changes) provide clear medical evidence of the cause of death.
- A proper postmortem report strongly supports that death was due to a medical cause, not negligence, and protects doctors in court.
- Not advising postmortem may appear as if the doctor is trying to avoid a negligence inquiry.

✅ Message to doctors:

If a death in hospital is sudden and unexplained, make it an MLC and request postmortem. This is the safest way to protect yourself from false allegations of medical negligence.

Doctor on Leave & Patient in Nursing Home



The Common Practice

In many nursing homes, patients are admitted and remain stable while the doctor may be out of station for emergencies. Senior nurses often conduct rounds and update the doctor over the phone.



The Legal Problem

This practice is legally incorrect. If a stable patient suddenly becomes sick and dies, and the patient's family files a case, it is very difficult to explain in court why no qualified doctor saw the patient that day. This amounts to gross medical negligence.



What the Law Requires

The law clearly mandates that a qualified doctor must examine the patient in the absence of the primary physician. Even if the nurse is experienced and the treatment plan does not change, legally it is negligence if no doctor examines the patient.



Safe Practice for Doctors

If you are going on leave, make arrangements with a local doctor friend or colleague to conduct formal rounds. The primary physician can take leave, but failing to arrange a qualified substitute is considered negligence.



Commitment to Safety

Always ensure that a qualified doctor sees your patients in your absence. This protects the doctor from medico legal complications

Medico Legal Tip:

Does Writing ‘Not Valid for Legal Purpose’ Really Protect You? 🤔

Understanding the Legal Implications of Medical Disclaimers ⚖️

▶ The Trend: Adding ‘Not Valid for Legal Purpose’

Recently, it has become common for doctors to include a disclaimer at the bottom of certificates and reports stating, ‘Not valid for legal purpose’. But does this actually offer any real protection? 🤔

📖 What Does the Disclaimer Mean?

Many doctors now add this line to medical certificates, radiology reports, or lab results, believing it will shield them from legal responsibility. However, here’s the reality — this statement has no standing in law. ❌

⚖️ Legal Reality: No Protection

Any medical document you issue — whether a certificate or a report — is by default considered valid for legal purposes if submitted to a court or relevant authority. Simply adding the phrase ‘not valid for legal purpose’ does not absolve you from legal responsibility. If a certificate or report is inaccurate, the issuing doctor can still be held liable. 🏛️

✅ What Truly Protects You?

- 📝 Ensure every document is accurate
- 📁 Maintain thorough and proper documentation
- 🧑‍⚕️ Learn the correct format for legally valid medical certificates

These practices, rather than misleading disclaimers, are what truly protect you within the legal system.

🚫 The Truth About Disclaimers

Disclaimers like ‘not valid for legal purpose’ are meaningless in the eyes of the law. It’s essential to commit to safe and ethical medical practices for true legal protection. 🙏

Medico Legal Tip: Legible Handwriting in Prescriptions

Legal Mandate

Legible handwriting in medical prescriptions is not just a norm, it is now a legal requirement and a patient's right. Courts across India have emphasized that patients must be able to read and understand their prescriptions. Recently, the Punjab and Haryana High Court declared that clear prescriptions are part of the fundamental right to health under Article 21 of the Constitution.

Court Observations

In one case, Justice Puri found a medico legal report completely illegible during a bail hearing. This prompted suo motu proceedings on the larger issue of legibility in medical documents. The Court observed that illegible handwriting can cause serious harm, create confusion, and deny patients the ability to give informed consent. Other High Courts, including Orissa, Uttarakhand, and Allahabad, have also raised similar concerns. In fact, the Allahabad High Court imposed a fine on a doctor in 2018 for shabby handwriting in a medico legal report.

Risk for Doctors

I am sure that The day is not far when courts may hold illegible handwriting itself as medical negligence. If a patient receives the wrong medicine or a pharmacy dispenses the wrong drug due to unclear handwriting, and complications occur, doctors will be held responsible. Most courts have already issued warnings, and if a case is filed, they are likely to come down heavily on doctors who fail to write legibly.

Message to Doctors

Doctors must ensure that prescriptions are written clearly. This is not just a request but a legal mandate and a patient's right. Writing legibly protects patients from harm and safeguards doctors from allegations of negligence.

Medico Legal Tip: Follow Up Dates & Their Legal Importance

The Issue

Many patients require follow up for serious conditions such as retinopathy of prematurity, tapering steroids, or follow up imaging. Often, they fail to return on time and later present with complications at an advanced stage. When outcomes are poor, families frequently blame doctors for medical negligence.

⚠️ Legal Risk

There are several cases where patients continued prolonged medications like Wysolone or Methotrexate without follow up, leading to serious adverse events. While doctors usually explain follow up verbally, the legal mistake is not writing the follow up date in the discharge summary. In consumer court, patients often claim they were not informed and argue that no counselling was done. Without documentation, doctors have no proof that the patient was advised to return for follow up.

📌 The Legal Protection

What protects doctors in such cases is written documentation of the follow up date in the discharge summary. This serves as proof that the patient was advised properly and shifts responsibility back to the patient if they fail to comply.

📖 Decided Judgments

- In one case, an ophthalmologist was not held negligent even though the patient lost complete vision following prolonged topical steroid use, because documentation of follow up after 7 days was recorded in the discharge summary.
- In contrast, a paediatrician was held negligent and ordered to pay ₹76 lakh compensation because of failure to write proper retinopathy of prematurity follow up dates and advice in the discharge summary, which led to blindness.

✅ Message to Doctors

Always take time to write discharge summaries carefully. Do not treat them casually. A discharge summary is not just a medical note — it is a legal document that can safeguard you in court. Specifically mention the follow up dates in the discharge summary. Remember, your oral counselling to come back will not stand in court if you have not documented it in the case sheet.

Let's commit to legal safety in medical practice by documenting follow up dates clearly.

Professional Jousting on Social Media: A Emerging Risk for Doctors

Just go through the screenshot which I have shared before reading the message.

📝 The Issue

In today's digital age, doctors increasingly use social media to share knowledge, highlight challenges, and showcase life saving efforts. Unfortunately, a disturbing trend has emerged: fellow doctors publicly criticizing and alleging negligence in comment sections, without knowing the full clinical context.

- Even the Supreme Court has said that detailed analysis must be made before coming to a conclusion. Unfortunately, many doctors are commenting harshly without knowing the ground realities.

⚠️ Legal Risk

- Public comments can instigate patients or attendants to file complaints or legal cases.
- Remarks such as “RIP protocol” , “hospital should be shut down” or “ Protocols are not followed “ may be interpreted as evidence of negligence, especially when made by doctors.
- Once such cycles of blame begin, any doctor can become the next target.

📌 Legal Protection

- Constructive critique should remain within professional forums, peer review committees, institutional audits or even personal messages directly.
- Avoid judgmental remarks in public comment threads.
- Do not pass loose comments about colleagues on social media without knowing the reality.
- Protect colleagues today to safeguard yourself tomorrow.
- Remember: casual or speculative comments have no evidentiary value but can still trigger unnecessary litigation.

✅ Message to Doctors

Doctors must restrain from public jousting on social media. Let us maintain unity, uphold dignity, and safeguard the profession from avoidable medico legal risks.

📷 Note

I have shared one screenshot here as an example. There are thousands of similar comments on social media where doctors criticize other doctors publicly.

⚖️ **Medico Legal Tip: Non MLC Later Turns Out to be an MLC**

📝 The Issue

Patients may be admitted as non MLC cases (e.g., accidental falls, unconscious states). Later, when they regain consciousness or disclose new facts, they may reveal a history of assault or suspected crime.

⚠️ Legal Risk

Failure to convert such cases into MLCs can expose doctors to allegations of suppression, negligence, or complicity.

Legal Protection

The law permits late MLC intimation. As soon as suspicion of crime arises, even after several days, doctors are legally bound to inform the police.

Example Case

A female patient is admitted with a fracture, initially reported as a self fall at home. After two days, she discloses that domestic violence occurred. In such a situation, the doctor must make the MLC on the day she disclosed the violence, not retrospectively. The intimation should carry the actual date and time when the disclosure was made.

Message to Doctors

Do not fear late MLCs. They are legally valid. The safeguard is clear documentation: record the actual date and time when suspicion arose, and note in the case sheet that the MLC was made late because initially it was not considered an MLC. Never alter dates or issue retrospective intimation.

Medico Legal Tip: Was IPC Safer for Doctors Than BNS?

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Is the Bharatiya Nyaya Sanhita (BNS) more doctor-friendly than the Indian Penal Code (IPC) in terms of protecting medical professionals from criminal prosecution?

Legal Risk

Under IPC 304A, medical negligence was treated as general negligence. There was no clear definition, no clarity on which registered practitioners were covered, and courts had full flexibility to give only a fine, or a fine plus imprisonment. This made IPC less harsh and far more forgiving for doctors.

Under BNS Section 106, things have changed:

- Medical negligence is clearly defined
- Registered practitioners are specifically identified
- Imprisonment of 2 years is mandatory if negligence is proven.

Just Google “BNS Section 106” — you will understand in detail how strict it has become.

✅ Message to Doctors

BNS has clearly become tougher on doctors. Laws are getting stricter year after year. There is no shortcut anymore — only strong medico legal knowledge and disciplined practice can protect you.

⚖️ Medico Legal Tip: Can Allopathic Doctors Prescribe Ayurvedic Medicines?

📝 The Issue

An allopathic doctor in Delhi was recently penalized in consumer court for prescribing an Ayurvedic medicine (Liv 52) to a patient admitted with suspected dengue fever. The patient later died, and during case sheet review, it was found that the doctor had prescribed an Ayurvedic hepatoprotector despite not being qualified in Ayurveda.

⚠️ Legal Risk

The consumer court held this to be medical negligence, stating that an allopathic doctor is not legally permitted to prescribe Ayurvedic drugs and Compensation was ordered.

This case reinforces a critical medico legal principle: cross pathy is prohibited.

📌 Legal Protection

- Allopathic doctors are legally allowed to practice only modern medicine and surgery.
- Prescribing Ayurvedic, Homeopathic, Siddha, Unani, or any alternative medicine without qualification is prohibited under NMC rules.
- We have no formal training in their mechanism, interactions, or adverse effects.
- If a complication occurs, courts will hold the doctor negligent for practicing outside their qualification.
- The argument that “other systems prescribe allopathic drugs” has no legal value. As they are not governed by NMC and One wrong does not legalise another wrong.

✅ Message to Doctors

Mixing medical systems may feel harmless, but legally it is negligence.

Always practice strictly within your qualification and avoid prescribing any medicine outside the modern system of medicine.



Medico-Legal Tip:

Belongings of a Patient in a Medico Legal Case



The Issue

In medico legal cases, doctors naturally focus on treatment — but the patient's belongings often hold crucial evidence that can influence the entire investigation.



Legal Risk

If clothes, documents, or personal items are misplaced, thrown away, or not handed over properly, it can break the chain of custody and lead to allegations of negligence or evidence tampering against the hospital or doctor.



Legal Protection

Always collect all belongings — clothes, paper slips, objects found in pockets, or any material present on the patient's body.

Place them in a sealed box or bag, label it with the patient's name and hospital ID, document it clearly in the MLC register, and hand it over to the investigating officer with a proper acknowledgment.



Message to Doctors

When an MLC patient arrives — whether due to a stab injury, assault, or suspected homicide — treat every belonging as potential evidence. Clothes with blood stains, papers, or personal items must be preserved exactly as received.

What I suggest as a medico legal advisor is — follow a systematic, documented process for preserving and handing over belongings. These small steps protect you and help deliver justice to an innocent victim

Medicolegal Tip: Posting Patient Videos in Reels: Always Take Written Consent

The Issue

In today's digital era, many doctors post reels to showcase their achievements and treatment results. But many of these videos unintentionally reveal the identity of patients

Example :

“Many reels today show emotional moments after an IVF delivery — the mother crying with joy,, the couple thanking the team, or the doctor announcing the successful birth. In that moment of excitement, the parents may happily say ‘Yes, doctor, you can record this.’

But later, when the excitement settles, they may feel uncomfortable that such a private and sensitive part of their IVF journey — including the delivery moment — has been shared publicly. They may then file a complaint saying their privacy was breached.

Legal Risk

The Medical Council guidelines are clear:

If a patient's identity is revealed in any form, written consent is mandatory.

Legal Protection

- Take written consent before recording or posting any reel involving a patient.
- If the patient's identity is visible, written consent is compulsory.
- If the patient's identity is fully hidden, consent is not required

Always take written consent before involving patients in reels or videos, even if they appear comfortable at that moment.

Patient privacy is non-negotiable. Written consent is your legal shield.

Medicolegal Tip: Never Issue a Medical Certificate Without Examining the Patient

The Issue

Some doctors issue medical certificates based on phone calls or attendants' requests — but this is a serious medico-legal violation that can lead to disciplinary action.

Legal Risk

In one real case, a doctor issued a certificate purely on an oral request from the patient's attendants. Later, it was discovered that the patient was actually partying in the USA on the exact dates mentioned in the certificate.

Because of certain reasons, the validity of the certificate came into question, and the case was referred to the Delhi Medical Council for inquiry which held the doctor guilty of issuing a certificate without examination and suspended his registration for two months.

Certificates issued without examination can be misused for insurance, employment, leave, or legal matters — and the doctor becomes fully liable.

Legal Protection

A medical certificate is a legal document. You must personally examine the patient before issuing it. If challenged, failure to examine is treated as negligence or professional misconduct. Never issue a certificate unless you have examined the patient yourself.

Medicolegal Tip: Take a Request Letter Before Issuing Any Medical Document

The Issue

Doctors often issue certificates, discharge summaries, or medical records based on oral requests from patients, relatives, police, or other authorities. The common mistake is not taking a written request letter.

Legal Risk

If a document is issued without a written request, the patient may later claim that their medical information was released without consent. Medical councils and courts take such privacy breaches seriously and may hold the doctor responsible.

Legal Protection

- Take a written request letter for every medical document you issue.
- If the patient is requesting, their signature serves as consent.

- If a third party is requesting, ensure the patient also signs the request letter.
- Never issue any medical document based only on an oral request.

Decided Judgments

Courts have repeatedly held that releasing medical information without documented consent amounts to breach of confidentiality and professional misconduct.

A simple written request letter protects you legally and ensures patient privacy is respected.

Medicolegal Tip: **Can a Doctor Question the Opinion of an Expert Committee in Court?**

The Issue

When a complaint of medical negligence is made, courts or authorities often form an expert committee to review the case. Many doctors believe that once the committee gives its opinion, it cannot be questioned. Legally, this is incorrect.

Legal Risk

If the committee does not include a specialist from the concerned field, its opinion may be considered incomplete or unreliable. For example, if the allegation is in Paediatrics, at least one paediatrician must be part of the committee.

Law clearly states that negligence must be judged by peer doctors from the same specialty.

Legal Protection

Doctors have the right to challenge the validity of an expert committee if:

- It does not include a specialist from the relevant field
- The members lack the required expertise to judge the case

This ensures that the medical facts are properly understood and fairly evaluated.

Whenever a case is filed against you, request that at least one member of the expert committee belongs to your specialty. This protects you and ensures a fair, scientifically sound assessment.

Message to Doctors

You can question an expert committee if it lacks the right specialists. Fair evaluation is your right.



MEDICO-LEGAL TIP : The 6 Crore Lesson Every Doctor Must Know



The Issue

“₹6,00,00,000 crores compensation. Yes, Dr you heard it right — 6 crores.”

This was the compensation awarded in the famous Kunal Saha case.



Legal Risk

Do you know the key reason because of which negligence was established?

“It was excessive administration of drug — beyond the recommended dosage limits in medical literature.”



Reality Check

“As doctors, we often adjust doses of drug especially which have Linear Dose-Response based on our experience, clinical judgement and patient response because individual patient respond differently for different dosage. With the intention to benefit the patient.

But the law does NOT accept personal experience as evidence.”



Warning

“If you cross the dosage limits written in textbooks or guidelines without any evidence of literature supporting your action, it can be treated as medical negligence — even if your intention was to help.”



Decided Judgment

Kunal Saha case stands as a reminder that deviation from recommended doses without literature support can attract massive liability.



Legal Protection

“What I suggest as a medico-legal advisor is this — strictly adhere to recommended doses.

Your individual experience cannot defend you in court, only textbook literature or guidelines can save you, so be careful when you deviate from the standard dosages.”



MEDICO-LEGAL TIP: The Myth of “Not Valid for Legal Purpose”



The Issue

“Writing ‘Not Valid for Legal Purpose’ is the most useless thing a doctor can do.”

A new trend has started—doctors are writing ‘Not valid for legal purpose’ on certificates, radiology reports, even blood reports, thinking this one line will protect them.

Recently, a patient used such a certificate to avoid appearing in court.

When questioned, the doctor said, “But I wrote not valid for legal purpose!”

The court rejected this completely.

⚠️ Legal Risk

Under the Indian Evidence Act, any document signed by a registered medical practitioner is automatically a legal document.

Your disclaimer has zero legal value.

📌 Consequence

“Remember—if the information is inaccurate or fake, you are liable, regardless of what you wrote at the bottom.”

📖 Legal Truth

A disclaimer cannot override the law.

📌 Core Message

“Don’t rely on a false safety sentence.

Disclaimers don’t protect you — accuracy, proper examination, and correct documentation do.”

“Stop practicing with myths; start practicing with the law.”

Medicolegal Tip: ⚖️ Can a Surgeon Take Consent for Anaesthesia?

📖 The Issue

In many hospitals, surgeons take anaesthesia consent because anaesthetists arrive only at the time of surgery. But legally, this is a problem. Consent requires explaining the procedure, risks, and alternatives — and only an anaesthetist is qualified to explain anaesthesia related risks.

⚠️ Legal Risk

If a surgeon takes anaesthesia consent, courts may consider it as invalid, because the surgeon is not the expert in anaesthesia. This can lead to allegations of improper consent, negligence, or failure to disclose risks.

📌 Legal Protection

- Anaesthesia consent must be taken only by the anaesthetist.
- It should be countersigned by the anaesthesiologist who is giving the anaesthesia.
- Surgeons should avoid taking anaesthesia consent, as they cannot explain anaesthesia specific risks and alternatives.

Decided Judgments

Courts have repeatedly held that consent must be taken by the doctor who is performing the procedure or administering the intervention — including anaesthesia.

Anaesthesia consent is the legal responsibility of the anaesthetist, not the surgeon.

What should a doctor check before buying professional indemnity insurance?

1 Check the AOA : AOY Ratio — This decides how much you ACTUALLY get
 “Your policy may say 1 Crore, but if the AOA:AOY ratio is 1:2, the company will pay only 50 Lakhs for a single claim.
 If the court awards 80 Lakhs, you will pay the remaining 40 Lakhs from your own pocket.”
 “Most doctors don’t even know this ratio exists — but this is the clause that destroys them financially.”

2 Check if Vicarious Liability is Covered

“Many policies cover only the doctor.
 But in real life, you will be sued even if the mistake is done by a nurse, technician, or junior, because of the legal principle of Vicarious Liability.”
 “If your policy doesn’t cover vicarious liability, you are standing naked in the courtroom.”

3 Check if Legal Defense Costs Are Included

“Hiring a good medico legal advocate today can cost more than the compensation claim itself.
 If your policy doesn’t cover legal defense, you will burn lakhs before the case even starts.”

4 Final Message

Indemnity insurance is your last line of defense.
 Your first line of defense is preventing cases by learning the law.”
 “Attend medico legal masterclasses specially designed for doctors.

Visit the website WWW.MEDICOLEGALAID.COM to register and learn the legal issues concerned with medical practice that can save your career.”

Medico Legal Tip: What Doctor MUST Do When a Consumer Case Is Filed

1 Inform Your Indemnity Insurance Immediately

- The moment you receive a legal notice, inform your Professional Indemnity Insurance company.
- Delay in intimation is one of the biggest reasons for claim rejection.

2 Never Pay the Patient in Cash

- Even on humanitarian grounds, never pay in cash.
- Cash has no proof, no evidence, and no legal value.
- If you ever help, do it only by cheque or online, so the record protects you.

3 Respond to the Notice Within Time

- Consumer courts can pass ex parte orders.
- If you don't appear or don't respond, the case will continue without you — and the final judgment will be delivered without hearing your side.
- Always respond within the stipulated time.

When an Unconscious Patient Has No Attenders — Including Cases Where Attenders Abscond After Admission

The Issue

A patient is unconscious, requires an urgent life saving procedure, and either:

- No attenders are available, or
- Attenders admit the patient and then abscond.

Legal Risk

If a single doctor proceeds alone, later complications may lead to allegations of:

- Performing a procedure without consent

- Acting without due diligence
- Causing harm without proper authorization
- Not attempting to contact relatives

Legal Protection

In emergencies, the law recognises implied consent, but courts expect collective, documented decision making.

 Form a Medical Board immediately.

This typically includes:

- Medical Superintendent / Hospital Director
- Two or more senior consultants from relevant specialties

The Board must:

- Confirm the urgency, Agree unanimously, Document the need for the procedure
- A properly documented Medical Board decision is legally treated as valid consent.

Decided Principles

- A Medical Board decision carries far greater legal weight than a single doctor's note
- Hospitals must record attempts to contact relatives
- Even if the surgical outcome is not favourable, the operating doctor remains in a legally safer position because the decision was collective, transparent, and taken in the patient's best interest.

Medicolegal Tip: Known Surgical Complications Are Not Negligence

The Issue

A 6-year-old child underwent surgery for congenital ptosis.

Post-surgery, ptosis worsened from moderate to severe and vision dropped from 6/9 to 6/18.

The family sued the doctor for ₹15 lakhs alleging “lack of cure”.

Legal Risk

Doctors often assume that a bad outcome = negligence.

But in law, outcome is never the test.

Courts require proof of deviation from standard practice — not proof of deterioration.

Legal Protection (Supreme Court Principles)

The Supreme Court (Neeraj Sud vs Jaswinder Singh, 2024) clearly held:

- A patient's condition may worsen after surgery.
- This does NOT automatically mean negligence.
- A bad outcome does not prove the surgery was wrong.
- Post-operative deterioration alone is NOT evidence of negligence.
- Rare complications can occur even when the doctor has done everything correctly.
- If the doctor follows accepted protocols, he cannot be held liable just because the surgery failed.

The Court restored the dismissal of the complaint and held no negligence.

Decided Judgment

Neeraj Sud & Anr. vs Jaswinder Singh (Minor) & Anr., Supreme Court of India, 2024

A landmark ruling reaffirming that known complications ≠ negligence.

Message to Doctors

Document pre-operative counselling, explain known complications, and record that outcomes can vary.

A complication is not negligence — lack of documentation is.

Medico-Legal Tip: Can Doctors Video-Record the Consent or Counselling Process?

The Issue

Doctors often ask whether they are legally allowed to video-record consent or counselling, especially before major surgeries.

Legal Risk

Lack of proof of proper counselling is one of the most common reasons for allegations of “no informed consent” in medico-legal cases.

Legal Protection

- Yes — doctors can legally video-record the counselling or consent process.

- It is especially advisable for major, life-changing surgeries such as: ENUCLEATION, AMPUTATION, REMOVAL OF GENITAL ORGANS, TRANSPLANTS, ANY HIGH-RISK OR HIGH-COST PROCEDURE
- The patient must be informed that the recording is being made.
- Privacy and confidentiality must be fully protected.
- The recording must be stored securely and never shared publicly.
- The video becomes strong evidence that proper counselling was done.

Message to Doctors

Video-recording consent is medico-legally safe, ethical, and highly protective — especially when future disputes are likely.

Hospitals Are Vicariously Liable — Courts Again Support Doctors

The Issue

When negligence is alleged, hospitals often try to shift the entire blame onto the treating doctor. Many doctors fear they will be left alone to face compensation claims.

Legal Risk

If doctors are made the sole accused, they may face: Personal compensation liability, Criminal prosecution, Professional harassment, Reputation damage.

Legal Protection

Recent judgments reaffirm that hospitals cannot escape liability for negligence committed by doctors working under them. Courts have held that hospitals are vicariously liable because:

- They earn from the doctor's services
- Patients come to the hospital as an institution
- Hospitals provide integrated care
- Ownership (medical or non-medical) does not change liability

Decided Judgments

- Maharaja Agrasen Hospital vs. Master Rishab Sharma (2016)
- Pooja Sharma vs. Maharaja Agrasen Hospital (2019)
- Uttarakhand High Court (2024) — Hospitals cannot distance themselves from negligence by their employed doctors.

These rulings clearly state that hospitals must share accountability and compensation.

✅ Message to Doctors

You are not alone in medico-legal cases.

If the hospital earns from your work, the hospital must also share the liability.

Courts consistently protect doctors from being unfairly targeted.

⚖️ **Medico-Legal Tip: Financial Assumptions Can Destroy Your Career**

💡 The Hidden Mistake Doctors Don't Realise They're Making

One of the biggest medico-legal errors happens before a case ever reaches the court — it happens when a doctor assumes the patient cannot afford a particular treatment. Many doctors avoid mentioning costly options like biologics, ECMO, organ transplants, or advanced interventions because they don't want to burden the family. It feels sympathetic in the moment, but legally it is extremely dangerous.

🏛️ What Happens in Court

When a patient dies, the same relatives often say:

“Doctor never told us about this option. We would have sold our house to save him.”

In the eyes of the law, that single statement is enough.

Failing to inform the patient about the best available treatment option — even if it is expensive — is treated as medical negligence, and courts have repeatedly awarded compensation in such cases.

👩‍⚕️ Your Duty as a Doctor

Your responsibility is to offer the scientifically best option, not the financially cheapest one.

Never assume the patient's financial capacity.

Give every option — the ideal, the alternative, and the affordable.

Let the family decide.

Let your documentation protect you.

🔒 Your Safety Lies in Full Disclosure

Your duty is medical.

Their decision is financial.

Your protection comes from complete, transparent communication.

Medico-Legal Tip: Documentation Check Before Handing Over a Case Sheet

 Before giving a case sheet to the Court or Police, doctors must do a strict documentation audit.

This single step can decide whether your defence stands strong or collapses.

1 First — verify all consent forms.

Make sure every consent is cross-signed by you.

If the doctor's signature is missing, the consent can be treated as invalid, and this mismatch can seriously weaken your defence.

2 Second — ensure every clinical step is documented clearly and chronologically.

Record all actions taken to save the patient, including exact drugs used, doses, timings, and interventions.

Incomplete documentation is interpreted as “not done”.

3 Third — document all cross-specialty consultations.

Write the name of the consultant, specialty, date, and time.

Written proof of cross-consultation shows that you took timely expert help and did your best for the patient.

 Before handing over the case sheet, check these three things without fail.

Good documentation is not paperwork — it is your legal shield.

Medico-Legal Tip: 3 Mandatory Consents Before Any Major Surgery

? What is the minimum consent a doctor must take before doing major surgery?

Many doctors still believe one “general consent” is enough.

Legally, that is wrong — and dangerous.

1 Surgical Consent

Taken by the operating surgeon.

Explain the procedure, the possible complications, and the available alternatives.

This is your primary legal shield.

2 Anaesthesia Consent

Taken by the anaesthetist.

Explain the type of anaesthesia, its risks, and possible side-effects.

Courts treat this as a separate responsibility.

3 Blood Transfusion Consent

If there is even a possibility of needing blood, take a clear, written approval.

Explain transfusion risks and document the family's understanding.

Never Merge These Consents

These three consents must always be separate.

A combined or pre-printed “one-page consent” is legally weak and may be treated as invalid.

Your Safety Tip

Clear, separate, procedure-specific consents protect you in court and show that you followed the standard of care.